DATE: July 11, 2025
SUBJECT: Allegations of Regulatory Noncompliance, Invalid Policy Implementation, and Constructive Adverse Actions at Phoenix Indian Medical Center (PIMC), Indian Health Service (IHS)

I. INTRODUCTION

This disclosure is submitted pursuant to **the Inspector General Act** and follows the closure of my prior complaint to the U.S. Office of Special Counsel (**OSC Case No. DI-25-001642**) without referral. I request that the HHS Office of Inspector General investigate serious and continuing violations of federal policy, governance obligations, and personnel rules at the **Phoenix Indian Medical Center (PIMC)**, part of the Indian Health Service (IHS).

The misconduct detailed herein is institutional in nature, implicates public health risk, and includes improper removal of official duties and responsibilities without process or documentation.

II. REGULATORY AND POLICY VIOLATIONS

1. **Invalid Policies – Failure to Follow Required Approval Procedures**
   As of June 2025, at least **47 clinical and operational policies** in effect at PIMC were implemented or renewed **without complying with the Hospital Policy Management Policy** then in force. Specifically, the required steps for review and approval were bypassed:
   - Policies were not properly routed through or reviewed by the designated **Policy & Procedures Officer** (PX3147).
   - Policies were not formally **ratified by the Governing Body**.

These deviations render the affected policies **procedurally invalid**. They are being used to govern patient care, risk management, and staff accountability without legal or regulatory standing. Multiple written warnings and memoranda issued by me to Executive Leadership and the Compliance Officer have been ignored.

2. **Absence of a Sentinel Event Reporting Policy**
   PIMC continues to operate without a compliant **Sentinel Event Reporting Policy**, in violation of **42 C.F.R. § 482.21** and **CMS Conditions of Participation**. Despite internal awareness of this regulatory deficiency, leadership has declined to adopt or promulgate a policy aligned with CMS definitions and incident escalation thresholds, undermining patient safety and required response standards.

3. **Noncompliance with Indian Health Manual – Part 1, Chapter 1 (TN 24-16)**
   The October 2024 **Transmittal Notice 24-16** requires IHS facilities to maintain:
   - A formally published policy inventory
   - Properly ratified Governing Body bylaws
   - Defined approval channels for facility-wide policy
   - Internal policy authorities consistent with national standards

PIMC remains out of compliance with these directives. The governing framework lacks ratified documents, approved procedures, and internal accountability mechanisms. My attempts to bring the facility into compliance were obstructed.

III. OBSTRUCTION AND CONSTRUCTIVE ADVERSE ACTIONS

1. **Obstruction of Officially Assigned Duties (PD PX3147)**
   As the appointed **Policy & Procedures Officer (GS-0610-12)** for PIMC, I am responsible for overseeing policy development, coordinating regulatory compliance, and supporting the Governing Body. Since early 2023, I have been:
   - Excluded from all policy review processes
   - Denied access to regulatory platforms
   - Prevented from performing required duties despite holding the formal position

These actions directly obstruct the execution of my federal position and compromise oversight of safety-critical operations.

2. **Constructive Suspension**
   I was placed on **administrative leave** without charges on **June 27, 2025**, following protected disclosures to OSC, CMS, GAO, and Congressional offices. On **July 11, 2025**, I was **verbally informed** that I had been transitioned to **investigative leave for 30 calendar days**.
   - I have received **no written notice**, **no proposed charges**, and **no opportunity to respond**
   - I remain entirely removed from duty, systems, staff, and assignments

These facts support a claim of **constructive suspension** under **5 U.S.C. § 7512(2)** and **MSPB precedent** (e.g., *Aguzie v. OPM*, *Brown v. DOI*).

3. **Constructive Demotion**
   Although my grade and pay remain unchanged, the **removal of all substantive duties associated with PD PX3147**, exclusion from all policy matters, and lack of access to perform core functions constitute a **constructive demotion** under **5 U.S.C. § 7512(3)**. No formal notice or process accompanied this change in function.

IV. TIMELINE OF EVENTS

- **March 21, 2023**: Filed sexual harassment complaint with IHS HQ → placed on administrative leave within hours
- **June 25–26, 2025**: Filed protected disclosures with OSC, CMS, GAO, HHS OIG, and members of Congress
- **June 27, 2025**: Placed on involuntary administrative leave without charges or due process
- **July 11, 2025**: Verbally notified of transfer to **30-day investigative leave** without written charges, proposals, or procedural rights

V. REQUEST FOR ACTION

I respectfully request that the HHS Office of Inspector General:

1. **Investigate** the improper implementation of invalid policies without procedural approval
2. **Examine** PIMC's failure to adopt required sentinel event reporting mechanisms
3. **Evaluate** the obstruction of my official responsibilities under PD PX3147
4. **Review** the lack of due process in placing me on investigative leave and stripping duties
5. **Refer** findings to CMS, the Departmental Appeals Board, or enforcement bodies as necessary to restore regulatory compliance and institutional accountability

VI. ATTACHMENTS

- Position Description: PX3147 – Nurse Consultant, Policy & Procedures Officer
- Disclosure & Retaliation Timeline (March 2023–July 2025)
- July 11, 2025 Verbal Notification of Investigative Leave (Declaration)

- OSC Referral Closure Letter – DI-25-001642

Respectfully submitted,
/s/ Kevin D. Turnage
Dated: July 11, 2025

ATTACHMENTS

Declaration of Kevin D. Turnage

RE: Verbal Notification of Placement on Investigative Leave

I, **Kevin D. Turnage**, declare under penalty of perjury that the following is true and correct to the best of my knowledge:

1. I am employed by the Indian Health Service (IHS), U.S. Department of Health and Human Services (HHS), and I serve in the official capacity of **Policy and Procedures Officer**, GS-0610-12, under **Position Description PX3147** at **Phoenix Indian Medical Center (PIMC)**.
2. On **June 27, 2025**, I was placed on **administrative leave** without a notice of proposed action, charge letter, or any documentation supporting the removal from duty.
3. On the morning of **July 11, 2025**, I received a **verbal notification** from an agency official—acting under the authority of the PIMC Executive Leadership Team—that my administrative leave status had been formally changed to **"investigative leave"** for a duration of **30 calendar days**, effective immediately.
4. As of the date of this declaration, I have **not received any written documentation** explaining the basis for the action, outlining the nature of the investigation, or affording me any due process protections under **5 U.S.C. § 7513** or related authority.
5. I have been fully excluded from all duties associated with my position, denied access to government systems and files, and barred from performing any aspect of my official responsibilities—effectively placing me in a **non-duty status** for over 10 consecutive business days.
6. I assert that the agency's failure to provide formal notification, explanation of charges, or opportunity to respond constitutes a **constructive suspension** within the meaning of **5 U.S.C. § 7512(2)**, and that the agency has also effectively implemented a **constructive demotion** by stripping me of my assigned responsibilities without lawful process.

I affirm under penalty of perjury under the laws of the United States that the foregoing is true and correct.

Executed on this 11th day of July 2025.

_____

Kevin D. Turnage
1003 E Pinto Ct
Gilbert, AZ 85296
kdturnage@protonmail.com
901-220-8664

U.S. OFFICE OF SPECIAL COUNSEL
1730 M Street, N.W., Suite 300
Washington, D.C. 20036-4505
(202) 804-7000

July 11, 2025

VIA ELECTRONIC MAIL
Kevin Turnage
kdturnage@gmail.com

Re: Final Decision on OSC File No. DI-25-001642

Dear Mr. Turnage:

The U.S. Office of Special Counsel (OSC) completed its review of the information you provided regarding your disclosure. After carefully reviewing the information you submitted, OSC has made a final decision to close your disclosure file. Please note that your prohibited personnel practice case, MA-25-004957, remains open and is currently under review.

You are a GS-12 Policy and Procedure Nurse Consultant with the U.S. Department of Health and Human Services (HHS), Phoenix Indian Medical Center (PIMC) in Phoenix, Arizona. In your disclosure, you allege that PIMC's lack of a sentinel events policy creates a danger to public health and violates 42 C.F.R. § 482.21, which defines the conditions of participation of HHS hospitals' quality assessment and performance improvement programs. Relatedly, you also allege that of 857 PIMC policies you reviewed, 31 are past due for review.

OSC is authorized by law to determine whether disclosures should be referred to the involved agency for investigation or review, and a report. OSC may refer allegations of violations of law, rule, or regulation; gross mismanagement; a gross waste of funds; an abuse of authority; a substantial and specific danger to public health or safety; or censorship related to scientific research. Disclosures referred to the agency for investigation and a report must include information sufficient for OSC to determine whether there is a substantial likelihood of wrongdoing. OSC itself does not have the authority to investigate disclosures and, therefore, does not conduct its own investigation.

We have insufficient evidence to support the referral of your allegations to the Secretary of HHS for investigation. During our conversation, you clarified that PIMC does have a sentinel event policy, albeit by a different name. You explained that the policy intended to cover sentinel events refers to such in broader terms as a reportable event. You admit that you are unaware of a law, rule, or regulation that specifically requires use of the term sentinel event, but you have concerns that a sense of urgency is lost by not using the term.

Unfortunately, the issues you raise concern matters of agency policy and discretion. And while you may strongly object to management's decision not to include the term sentinel

event, we are unable to find that the agency has acted outside its discretion in doing so. As for the past due policies for review, it is unclear whether the timely review of the policies violates a law, rule, or regulation or otherwise constitutes any other category of wrongdoing. Moreover, the overdue policies for review represent less than four percent of the total number of policies, which suggests that they are reviewed timely but for a few outliers. Given these factors, OSC will take no further action on your allegations.

Accordingly, OSC is closing your disclosure file.

Should you wish to further pursue your allegations, you may contact HHS's Office of Inspector General. More information for how to file a complaint online can be found here.

If you wish to discuss this matter, you may contact me at (202) 804-7028 or edrake@osc.gov.

Sincerely,

Emily M. Drake
Attorney
Retaliation & Disclosure Unit

DEPARTMENT OF HEALTH AND HUMAN SERVICES

OFFICE OF INSPECTOR GENERAL

OFFICE OF INVESTIGATIONS
SPECIAL INVESTIGATIONS BRANCH
330 INDEPENDENCE AVENUE, S.W.
ROOM 5409
WASHINGTON, DC 20201

May 30, 2025

By Email
Kevin Turnage
Kevin.Turnage@ihs.gov

RE: Whistleblower Complaint No. I210152

Dear Kevin Turnage:

The U.S. Department of Health and Human Services, Office of Inspector General, Office of Investigations (OIG/OI) has completed its review of your complaint of reprisal, which we received on May 28, 2025.

We have reviewed the reprisal allegations set forth in your complaint and determined that there is insufficient evidence to warrant further investigation. We base our determination on the following:

- ☐ **Complaint Filed in Another Forum** (*e.g.,* EEO, Federal/State Court, etc.)

- ☐ **Standing:** Complainant is not a protected whistleblower (e.g., an HHS contractor, subcontractor, grantee, subgrantee or personal services contractor, a member of the U.S. Public Health Service Commissioned Corps, an HHS employee or applicant for employment).

- ☐ **Protected Disclosure:** Complainant has not alleged, during the complaint nor during the intake interview, that they made or were preparing to make a protected disclosure, or that they were perceived as having made a protected disclosure under the relevant reprisal statute.

- ☐ **Personnel Action:** Complainant has not alleged, during the complaint nor during the intake interview, that an unfavorable personnel action was taken or threatened to be taken against them or that a favorable personnel action was withheld or threatened to be withheld from them.

This document, including any attachments and information contained therein, is the property of HHS OIG and is for OFFICIAL USE ONLY. The original and any copies of the report must be appropriately controlled and maintained. Disclosure to unauthorized persons without prior written approval of the Deputy Inspector General for Investigations or his/her designee is strictly prohibited and may subject the disclosing party to liability. Unauthorized persons may include, but are not limited to, individuals referenced in the report, contractors, and individuals outside the HHS. Public disclosure is determined by the Freedom of Information Act (Title 5, U.S.C., Section 552) and the Privacy Act (Title 5, U.S.C., Section 552a). *Version: 05/2023*

☐ **Knowledge:** The Complainant alleged that the management official(s) responsible for the personnel action had no knowledge of the protected disclosure(s) and did not articulate that the official perceived the Complainant as having made or prepared to make a protected disclosure.

☒ **Referral:** The Office of Special Counsel (OSC) is the agency of primary jurisdiction to investigate Federal employee reprisal complaints; therefore, you may wish to submit your reprisal allegations to the OSC. You may visit its web site at https://osc.gov for more information.

☐ **Complainant Voluntary Withdrawal:** You contacted our office stating your desire to withdraw your whistleblower complaint of reprisal. Finding no indication that your request to withdraw your complaint was in any way coerced, we hereby approve your request and dismiss your case.

☐ **Unresponsive Complainant:** 10-Day Letter issued to complainant with no response.

☐ **Other Reason** (if checked, please see reason below)

_____

After reviewing the complaint you submitted to the HHS OIG Hotline, SIB determined you are a federal employee. As a federal employee, the Office of Special Counsel is the agency that has primary jurisdiction to investigate federal employee reprisal complaints. As stated above, you may wish to submit your reprisal allegations to the OSC. Because OSC has primary jurisdiction in this matter, SIB has decided to close your complaint.

If your complaint contains non-reprisal allegations within OIG's jurisdiction (i.e., fraud, waste, or abuse), we will refer these allegations to the appropriate authority for any action that may be appropriate. The Inspector General Act and other laws prohibit the disclosure of the complainant without their consent unless OIG determines that disclosure is unavoidable during the course of an investigation. Such disclosures may occur when referring non-reprisal matters to other law enforcement agencies for investigation.

Sincerely,

Adam Layton

Adam Layton
Assistant Special Agent in Charge
Special Investigations Branch

Cc: Advice Branch, Office of Counsel to the Inspector General, OIG

POSITION TITLE: Nurse (Consultant)
ORGANIZATIONAL TITLE: Policy and Procedures Officer
PP/SERIES/GRADE: GS-0610-12
ORG LOCATION: DHHS, IHS, Phoenix Area, Phoenix Service Unit, Phoenix Indian Medical Center, Division of Quality Management, Performance Improvement
FLSA STATUS: Exempt
JOB CODE: PX3147

INTRODUCTION

This position is organizationally located in the Regulatory Compliance and Accreditation Department under the Division of Quality Management at the Phoenix Indian Medical Center (PIMC). This position serves as the Policy and Procedures Officer for hospital wide policies covering inpatient, ambulatory care, emergency and specialty areas. Incumbent serves as an analyst and advisor with the professional knowledge of a registered nurse who provides consultative and advisory services for the development, content, and authentication of the organization's policies and procedures, clinical protocols, and clinical practice guidelines

PIMC is a 129-bed medical center, a hospital based outpatient clinic, and three satellite clinics. Together these facilities serve over 7,600 Native American members of the local Tribal communities surrounding the greater Phoenix metropolitan area. In addition, PIMC serves as a Regional Medical Center and Referral Center that covers a three-state region (Arizona, Nevada, and Utah.) Each Tribal community is a stakeholder in the level of health care being provided.

The medical center provides inpatient and comprehensive ambulatory care, emergency medical, dental, eye care, environmental health, community health nursing, social services, preventive and behavioral health, with emergency care on a 24-hour basis, 7 days a week. PIMC has over 67,000 active clinical users, with over 3000 inpatient admissions/year and over 400,000 outpatient visits/year.

MAJOR DUTIES AND RESPONSIBILITIES

As the Policy and Procedures Officer, performs the following duties:

Analyzes organizational policies to assure that the content is valid, consistent, and without conflicting content assuring that the procedures describe actual or intended practice.

Stays abreast of changes in the various regulations and standards to develop new policies or revise current policies to ensure that the organization is compliant.

Develops and tracks new policies to meet the changing needs of the organization.

Oversees the coordination of the distribution of all new and revised policies.

Manages the on-line organizational policies and procedures.

Advises department managers and executive staff on ways to implement new regulations and processes.

Collaborates with management to maintain the various departments' scope of services.

Initiates, coordinates, and facilitates clinical and administrative multidisciplinary teams for the development and implementation of policies and procedures, clinical protocols, and clinical practice guidelines.

Research and advise on best practices to support organizational continual survey readiness

Provide guidance for developing and implementing competencies, internal logs and other forms of documentation that assist with meeting regulatory compliance and accreditation standards

Performs other related duties as assigned.

FES FACTORS

FACTOR 1 - KNOWLEDGE REQUIRED BY THE POSITON LEVEL 1-7

Professional knowledge and ability to apply a wide range of professional nursing/health care concepts, principles, and practices of advanced nature is required to sufficiently perform studies, advise management and develop policies and procedures, and carry out functions as the Policy and Procedures Officer.

Incumbent must possess and maintain a current, valid, and active state license/registration as a registered professional nurse.

Professional knowledge and expertise of a registered nurse to soundly apply knowledge and skill in a wide range of pertinent health care related regulations, such as the Joint Commission (TJC), Centers for Medicare & Medical Services (CMS), OSHA, Safety codes - State and Federal, clinical care standards and other applicable standards and guidelines for the medical facility. Additionally, detailed knowledge of the Service Unit, Phoenix Area, and IHS policy and procedures is required.

Knowledge in observing activities, processes, and demonstrations and then analyzing the information to develop policies for a complex health care environment.

Knowledge and skill in obtaining, analyzing, and selecting pertinent information, determining the most logical and effective sequence of the policy. Knowledge and skill in articulating complex information clearly, accurately, and thoroughly; apply consistent style, grammar and punctuation practices.

Knowledge and ability to consult with subject clinical and administrative experts to verify policy information.

Knowledge of medical literature search techniques.

Professional knowledge and expertise as a health care provider, performing clinical duties related to the nursing profession.

Broad medical knowledge and technical understanding of disease process, treatment standards of care and the comprehensive process of delivering health care services as described in the scope of TJC standards and chapters.

Knowledge and experience in medical staff organization and function and requirements.

Knowledge and skill in the use of word processing programs and inter and/or intranet.

FACTOR 2 - SUPERVISORY CONTROLS LEVEL2-4

The supervisor outlines overall objectives and available resources. The employee and supervisor, in consultation, discuss timeframes; scope of the assignment; including possible stages, and possible approaches.

As an expert, the incumbent works independently to plan and complete work assignments. The incumbent determines the most appropriate principles, and methods to apply in all phases of assignments, including the approach to be taken, degree of intensity, and depth of research in management advisories; interprets regulations on his/her own initiative, applies new methods to resolve complex and/or intricate, controversial, or unprecedented issues and problems, and resolves most of the conflicts that arise; and keeps the supervisor informed of progress and of potentially controversial matters.

The supervisor reviews work for soundness of overall approach, effectiveness in meeting requirements or producing expected results, the feasibility of recommendations, and adherence to requirements. The supervisor does not usually review methods used.

FACTOR 3 - GUIDELINES **LEVEL 3-4**

A wide variety of reference materials and manuals, including PSU, PAO, and IHS policies/procedures, standards from The Joint Commission and other regulatory agencies (i.e., OSHA, CMS) are available, but are not completely applicable to the work or have gaps in specificity. Further guidance comes from knowledge of disease processes, health care services processes, and evolving clinical practice standards. The incumbent applies considerable judgement in interpreting, adapting/modifying, researching, choosing, developing, and applying available guidelines for adaptation to specific problems or issues. The employee analyzes results and recommends changes.

FACTOR 4 - COMPLEXITY **LEVEL4-4**

Work consists of a variety of duties requiring the application of many different and unrelated clinical and administrative processes and clinical systems analysis methods to a broad range of Pl activities or to activities or to the in-depth analysis of PI issues. The incumbent makes decisions that involve major uncertainties with regard to the most effective approach or methodology to be applied. These changes typically result from continuing changes in service unit requirements or rapidly evolving technology in the health care industry. The work requires originating new techniques, establishing criteria, or developing new information.

FACTOR 5 - SCOPE AND EFFECT **LEVELS-4**

Independently interfaces with medical, nursing, management, and other staff to provide effective and approach policies and procedures, as necessary, when deficiencies and systems problems, or variation from the standards and processes are found. The work involves establishing criteria; formulating projects; assessing program effectiveness; or investigating or analyzing a variety of unusual conditions, problems, or questions. Activities affect PSU wide activities and are essential for fulfilling mission; achieving quality and safe patient care; TJC compliance; and hospital continual survey readiness.

FACTOR 6 - PERSONAL CONTACTS **LEVEL6-3**

Contacts are with multidisciplinary (medical staff, nursing staff, executive administration, and non-medical staff of the hospital) and interagency staff (headquarters, the Arizona PRO, CMS, and other agencies and organizations), as appropriate; and other representatives from various regulatory entities outside the agency in a moderately unstructured setting. Contacts are generally made face to face or by telephone and professional conduct is expected.

FACTOR 7 - PURPOSE OF CONTACTS **LEVEL7-C**

Contacts are to plan, coordinate, or advise on PI efforts, or to resolve issues or operating problems by influencing or persuading people who are working toward mutual goals and have basically cooperative attitudes. Contacts typically involve formalizing processes by writing policies andprocedures.

FACTOR 8 - PHYSICAL DEMANDS **LEVEL 8-1**

Work is sedentary. Some work may require walking and standing in conjunction with travel and to attendance at meetings and conferences away from the work site. Projects may demands the incumbent to work under stress, to meet short deadlines, or cope with changing priorities and multiple projects and tasks that are varied in nature. The computer work requires long periods of time using keyboard and a computer and CRT.

FACTOR 9- WORK ENVIRONMENT **LEVEL9-1**

Work area is adequately lighted, heated, and ventilated. The work environment involves everyday risks or discomforts that require normal safety precautions.

OTHER SIGNIFICANT FACTORS

The incumbent is required to maintain a current, valid, and state licensure, registration, and/or certification in his or her medical specialty.

Conditions of employment - this position requires, at a minimum, Level 5 - MBI moderate risk level since the contents of the PD meets the Public Trust position criteria.

Below is the **matching Factor Evaluation System (FES) Evaluation Statement** for Position Description PX3147 – *Nurse (Consultant), Policy and Procedures Officer, GS-0610-12*, aligned exactly with the **nine OPM FES factors** and using language and level designations provided in the PD.

FES Evaluation Statement

Position Title: Nurse (Consultant), Policy and Procedures Officer
Series and Grade: GS-0610-12
Position Number: PX3147
Organization: Phoenix Indian Medical Center (PIMC), IHS, DHHS

Factor 1 – Knowledge Required by the Position: Level 1-7

The position requires professional knowledge and ability to apply a wide range of advanced nursing/health care concepts, principles, and practices. The incumbent must possess:

- A current, valid, and active state license/registration as a registered nurse;
- Broad professional knowledge of clinical standards, care protocols, accreditation requirements (The Joint Commission), and federal regulations (CMS, OSHA, etc.);
- The ability to analyze, interpret, and apply regulatory standards to the development of institutional policies, procedures, and clinical guidelines;
- Skill in technical writing and editing, articulating complex medical and regulatory information in a clear, precise manner;
- Competence in inter-professional consultation and document authentication processes.

Point Value: 1250
Justification: Meets Level 1-7. Work requires professional knowledge equivalent to a Master's degree or a Bachelor's degree with significant specialized experience, and involves diverse clinical and regulatory responsibilities impacting institutional policy and care standards.

Factor 2 – Supervisory Controls: Level 2-4

The supervisor sets overall objectives and available resources. The incumbent independently plans and carries out assignments, determines technical methods, and makes authoritative decisions within the scope of duties. Completed work is reviewed for feasibility and adherence to mission requirements, but methods are rarely reviewed in detail.

Point Value: 450
Justification: Meets Level 2-4. The position functions independently, as an expert advisor and consultant, responsible for resolving complex policy and regulatory issues without close supervision.

Factor 3 – Guidelines: Level 3-4

Guidelines include Joint Commission standards, CMS regulations, OSHA codes, IHS and Area Office directives, and internal policy manuals. These materials are often incomplete, outdated, or lacking specificity. The incumbent must use judgment to adapt and apply guidelines to develop or revise internal policies and procedures.

Point Value: 450
Justification: Meets Level 3-4. Work requires extensive interpretation and adaptation of regulatory guidance to local conditions in a complex healthcare setting.

Factor 4 – Complexity: Level 4-4

The work involves a broad range of complex clinical and administrative issues. Assignments require evaluating the interrelationships of numerous systems, departments, and regulatory standards to develop integrated, institution-wide policy solutions. Work is highly analytical and involves judgment on methodology, policy gaps, and new regulatory trends.

Point Value: 225
Justification: Meets Level 4-4. Work is both procedurally and conceptually complex and requires innovative solutions in the context of dynamic health care regulation.

Factor 5 – Scope and Effect: Level 5-4

The work directly impacts the institution's ability to achieve regulatory compliance, maintain accreditation, and ensure patient safety. Work products (i.e., policies and procedures) affect all clinical and administrative departments and play a crucial role in readiness for external audits and inspections.

Point Value: 225
Justification: Meets Level 5-4. The position's outcomes have wide-reaching institutional impact, including legal, operational, and quality-of-care implications.

Factor 6 – Personal Contacts: Level 6-3

Contacts are with internal staff at all levels (clinical, administrative, executive), external stakeholders (CMS, Arizona PRO, accreditation entities), and representatives from

regulatory bodies. Interactions are unstructured and often sensitive or controversial in nature.

Point Value: 60
Justification: Meets Level 6-3. Contacts are extensive and involve negotiation, coordination, and expert-level engagement.

Factor 7 – Purpose of Contacts: Level 7-C

Contacts are for the purpose of advising, persuading, and negotiating the adoption of policy and procedural changes across multiple departments. The incumbent must influence senior management and multidisciplinary teams to implement changes and correct deficiencies.

Point Value: 180
Justification: Meets Level 7-C. Requires achieving compliance and policy alignment through professional influence rather than authority.

Factor 8 – Physical Demands: Level 8-1

The work is primarily sedentary. Occasional walking, standing, and travel to attend meetings is required. Work may involve stress due to competing priorities and deadlines.

Point Value: 5
Justification: Meets Level 8-1. Standard physical demands typical of administrative roles.

Factor 9 – Work Environment: Level 9-1

The work environment is an office setting with adequate lighting, ventilation, and safety. Risks are minimal and typical of a healthcare administration environment.

Point Value: 5
Justification: Meets Level 9-1. Standard office work environment.

Summary of FES Factors and Point Values

Factor	Level	Points
1. Knowledge Required	1-7	1250
2. Supervisory Controls	2-4	450
3. Guidelines	3-4	450
4. Complexity	4-4	225

Factor	Level	Points
5. Scope and Effect	5-4	225
6. Personal Contacts	6-3	60
7. Purpose of Contacts	7-C	180
8. Physical Demands	8-1	5
9. Work Environment	9-1	5
Total Points		**2850**

Grade Determination: GS-12

According to the OPM Grade Conversion Table for Professional and Administrative Positions, the total of **2850 points** falls within the GS-12 grade range (2755–3150 points).

DEPARTMENT OF HEALTH &. HUMAN SERVICES Public Health Service

Indian Health Service
Phoenix Indian Medical Center
4212 North 16th Street
Phoenix, Arizona 85016

MEMORANDUM

From: Hon. Kevin D. Turnage MSN Ed RN, NHDP-BC, AMB-BC, NPD-BC, NE-BC
Date: June 8, 2025
Subject: Legal Authorities Violated or Potentially Violated in Relation to the Policy and Procedure Nurse Consultant (PX3147) Role

I. FEDERAL STATUTES AND REGULATIONS

1. **42 C.F.R. § 482.12(a)(5)** – Accountability of hospital governing body for legal compliance and role assignments.
2. **42 C.F.R. § 482.21(b)** – Quality Assessment and Performance Improvement (QAPI) requirements and oversight responsibilities.
3. **42 C.F.R. § 482.22(c)(3)** – Responsibility for medical staff to develop and approve policies in line with bylaws.
4. **42 C.F.R. § 493** – Clinical Laboratory Improvement Amendments (CLIA) regulatory framework for lab quality systems.
5. **42 C.F.R. § 493.1251** – Requirements for documented test systems and quality review.
6. **45 C.F.R. § 160** – General administrative HIPAA provisions regarding compliance and enforcement.
7. **45 C.F.R. § 164** – HIPAA Privacy and Security Rules mandating organizational safeguards and documentation.
8. **45 C.F.R. § 164.530(i)** – Designation of a responsible official for policy oversight under HIPAA.
9. **44 U.S.C. § 3554** – FISMA obligations to appoint compliance officials to oversee IT security.
10. **44 U.S.C. § 3101** – Federal Records Act; requires proper documentation and preservation of decisions and policies.
11. **31 U.S.C. § 3729 et seq.** – False Claims Act; failure to properly vet policies may contribute to fraudulent claims.
12. **41 U.S.C. § 4712** – Whistleblower protections for reporting legal violations or gross mismanagement.
13. **5 U.S.C. § 2302(b)** – Prohibited personnel practices, including role reassignment in retaliation for protected disclosures.
14. **5 U.S.C. § 2302(b)(8)** – Specific protections for whistleblowers reporting violations of law, public health risks, or mismanagement.
15. **5 U.S.C. §§ 551–559** – Administrative Procedure Act; requires due process and reasoned decision-making in federal personnel actions.
16. **5 C.F.R. § 2635.101** – Executive branch ethics standards requiring impartiality and integrity in personnel management.

17. **29 U.S.C. § 218c** – Affordable Care Act (ACA) whistleblower protections specific to health care personnel.
18. **18 U.S.C. § 1513(e)** – Criminal retaliation statute protecting whistleblowers who report to federal enforcement bodies.
19. **29 C.F.R. § 1604.11(e)** – EEOC regulation on retaliation in connection with protected activity, including complaints.
20. **29 C.F.R. § 1960.46** – OSHA regulation protecting federal employees who report unsafe or noncompliant work environments.

II. AGENCY AND POLICY DIRECTIVES

21. **OMB Circular A-130** – Requires federal agencies to assign officials responsible for managing information, including policy systems.
22. **IHS Manual, Part 1, Chapter 1** – Establishes facility governance responsibilities and authorizes policy oversight positions like PX3147.
23. **IHS Manual, Part 1, Chapter 7** – Provides for whistleblower protections, grievance rights, and due process under IHS governance.
24. **TN 24-16** – 2024 Transmission Notice revising Part 1, Chapter 1, and retiring Chapter 5, reinforcing centralized compliance responsibility.
25. **HHS Instruction 930-1** – Department-wide standards of employee conduct emphasizing lawful execution of assigned duties.
26. **Executive Order 12731** – Federal ethical conduct principles requiring agency roles to be administered with impartiality and integrity.

III. ACCREDITATION AND INDUSTRY STANDARDS

27. **RC.01.01.01** – The Joint Commission standard requiring current, accurate policies accessible to those with compliance responsibility.
28. **LD.04.01.01** – Leadership structure standard mandating accountability for patient safety and operational governance.
29. **LD.04.01.05** – Requires senior leadership to establish and maintain policy development and approval mechanisms.